

14 DAY WAIT – URGENT REFERRAL**ROYAL STOKE UNIVERSITY HOSPITAL****General Enquiries: Tel 01782 671117****Clinical Enquiries: Tel 01782 675747****LUNG****ALL REFERRALS TO BE SENT VIA RAS (REFERRAL ASSESSMENT SERVICE) YOU ARE NO LONGER REQUIRED TO EMAIL/FAX UBRN**

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July 2017

PATIENT DETAILS				GP DETAILS			
Name:	Given Name Surname			Name:	Registered GP Title Registered GP Forenames Registered GP Surname		
Address:	Home Full Address (single line)			Address:	Organisation Name, Organisation Full Address (single line)		
NHS Number:	NHS Number			Phone No:	Organisation Telephone Number		
Hospital Number:	Hospital Number			Name of referrer:			
Date of Birth:	Date of Birth			Decision to refer date:	Short date letter merged		
Sex:	☐ Male ☐ Female			Interpreter required:	☐ Yes ☐ No		
Sign Language required:	☐ Yes ☐ No			Language:	Main Language		
Contact No (next 48 hrs):	Home:	Patient Home Telephone	Work:	Patient Work Telephone	Mobile:	Patient Mobile Telephone	

REASON FOR REFERRAL (please indicate as appropriate)

Urgent referral if signs/symptoms of superior vena cava obstruction or stridor – please contact the acute medical registrar on call immediately or refer directly to the emergency department.

☐ Unexplained haemoptysis aged > 40 years (Quantity / Duration)

CT chest scan suggestive of cancer

☐ Please note that patients in this category will have had a chest x-ray which has then generated the urgent CT scan as part of the UHNM Pathway for Suspected Lung Cancer☐ Chest x-ray suggestive of lung cancer (Please attach radiology report)

Date of CXR

Place of CXR

Please note that patients in this category will have a CT scan arranged to coincide with their initial OPA. Please ensure that your patient is aware that this will be the case.

☐ Chest x-ray suggestive of mesothelioma**Clinical Details / Additional Narrative (i.e. Synopsis of history)**☐ I confirm that I have discussed the possibility that the diagnosis may be cancer and they may wish to take a family member or significant other to the initial appointment.☐ I confirm that I have requested FBC U+E, LFT and GFR☐ I confirm that the patient is available to attend outpatients within the next 14 days

If patient is not available for the next 2 weeks, and aware of nature of referral, consider deferring the referral and seeing the patient again to reassess symptoms

Relevant Test Results (where available)

Full Blood Count (FBC)	Single Code Entry: FBC - full blood count	Liver Function Test	Single Code Entry: LFT - Liver function test
U&E	Single Code Entry: Urea and electrolytes level	eGFR	Single Code Entry: GFR (glomerular filtration rate) calculated by abbreviated

			Modification of Diet in Renal Disease Study Group calculation...
Smoking Status	SEE BELOW	Alcohol Status	SEE BELOW

Smoking Status

Smoking

Alcohol Consumption

Alcohol Consumption

Allergies:

Allergies

Medication:

Medication

Relevant Medical History:

Problems